



What significant complications can arise during pregnancy?

Pregnancy-induced complications are health issues that didn't exist before you were pregnant—and they will usually disappear after the birth of your baby. Throughout pregnancy, your doctor checks for signs of these complications. Some women have gynecological issues that can affect pregnancy, and sometimes problems arise with the placenta that is nourishing your baby.

Hypertension

» What is it?

It means you develop abnormally high blood pressure in pregnancy.

» What's the difference between pregnancy-induced hypertension and gestational hypertension?

There isn't a difference—both terms mean the same thing, that your high blood pressure in pregnancy is related. Your doctor took your blood pressure during your first appointment and will compare all subsequent blood-pressure readings to this one. In doing so she is making sure that over the course of your pregnancy there is no unexpected change—your blood pressure is expected to dip a little during pregnancy and then rise again to normal pre-pregnancy levels as you near full term. If at any point—and specifically after 20 weeks pregnant—it goes higher than 140/90 mmHg, you will be diagnosed with pregnancy-induced hypertension (PIH) or gestational hypertension (GH), depending upon the term your doctor chooses to use. The condition is subdivided into mild, moderate, and severe. If you have severe hypertension, this puts you at risk of a stroke, and you may have to have your blood pressure monitored up to four times a day.



Did you know...

- » **5 percent** of all pregnant women will develop some form of preeclampsia
- » **2 percent** of all pregnant women develop severe preeclampsia
- » **85 percent** of women with preeclampsia also have edema (fluid retention)
- » **0.5 percent** of women with preeclampsia go on to develop eclampsia

Preeclampsia

» What is it?

Preeclampsia is a condition brought on by pregnancy. It's characterized by hypertension plus protein in urine. You have a slightly increased risk of developing the condition if you are 40 or over, you are a teenage mom, you are overweight, or have a preexisting condition such as diabetes.

» My doctor says that I have pregnancy-induced hypertension. Does this mean I will get preeclampsia?

No, not necessarily. Although PIH can be a symptom of preeclampsia, it is not in itself a sign that preeclampsia is on its way. If you also have gestational proteinuric hypertension (protein in your urine), this is enough to diagnose mild preeclampsia, although other symptoms can lead to a diagnosis. If you also have one or more of the other symptoms, you are said to have moderate or severe preeclampsia. If you are diagnosed with any form of preeclampsia, your doctor will monitor your pregnancy very carefully over the remaining weeks, either with frequent office visits or at the hospital.

» If I have preeclampsia will I have to spend the rest of my pregnancy in the hospital?

That depends on how severe your preeclampsia is. If your preeclampsia is mild, you are likely to have to stay in the hospital for only a day or two while your blood pressure is monitored—usually up to four times a day. Often complete rest is all you need to get your blood pressure under control and you will be allowed to go home without further treatment. If you have moderate preeclampsia, your stay is likely to be longer and you are more likely to be given medication to bring down your blood pressure. You will probably also have a ultrasound to check that your baby is growing well. If your due date is still a while away, you may either be advised to stay in the hospital, or you will be closely managed as an outpatient, as long as your blood pressure has returned to normal and all seems well with your baby. If, however, you have severe early-onset

preeclampsia, you will probably have to stay in the hospital for the rest of your pregnancy so that you and your baby can be closely monitored. Your obstetrician may decide to deliver your baby early, although he or she will aim to deliver your baby no earlier than 37 weeks. If you develop preeclampsia of any severity after 37 weeks, you will usually be advised to have an early induction of labor.

» Symptoms

- » Frequent or permanent headache, usually above your eyes and over your brow.
- » Blurred vision, or “lightning flashes” in front of your eyes.
- » Pain in your abdomen or underneath your ribs.
- » Vomiting or general feelings of being unwell.
- » Reduced urination (called oliguria).
- » Sudden swelling in the extremities or face.

Eclampsia

» What is it?

This is a serious condition, characterized by seizures as well as all the symptoms of preeclampsia. In severe cases, eclampsia can lead to unconsciousness.

» Does having preeclampsia mean I will definitely develop eclampsia?

No. Only around one in every 200 women with preeclampsia will go on to develop full-blown eclampsia.

» How will eclampsia affect my pregnancy?

If you develop eclampsia your doctor will advise you to have your baby immediately. You are also at increased risk of placental abruption, when the placenta comes away from the wall of the uterus, and HELLP syndrome (see below).

HELLP

» What is it?

A severe form of preeclampsia, HELLP stands for Hemolysis (breaking down of red blood cells), Elevated Liver enzymes, and Low Platelet count.

» What does it mean for my pregnancy?

HELLP puts both you and sometimes your baby in significant danger, so you will be encouraged to give birth to your baby as soon as possible, regardless of how far along you are in your pregnancy. HELLP is more likely to affect the health of the mother than the baby, so your baby has a good chance of a positive outcome unless she is very small. Very low birth weight babies will be taken to the neonatal intensive care unit in order to give them the best possible chances of survival.